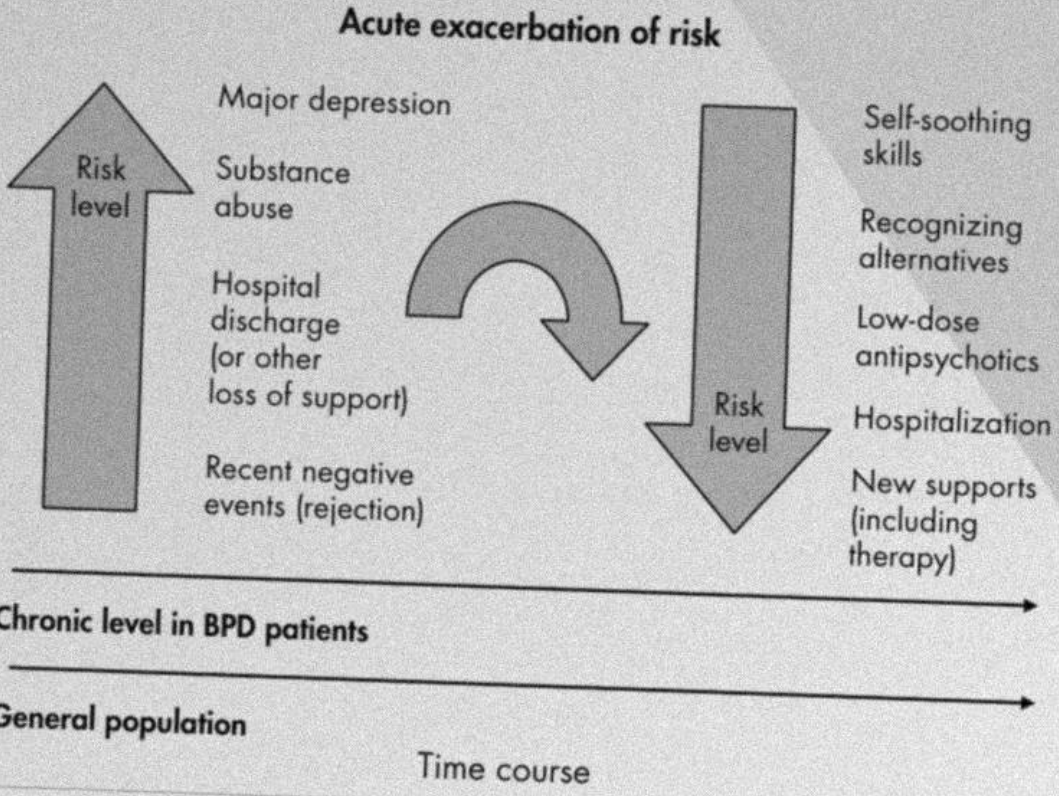


FIGURE 5–1. Acute-on-chronic suicide risk.
In patients with borderline personality disorder (BPD), the acute-on-chronic level of suicide risk (***curved arrow***) can change more quickly than in the general population and will be modified by several factors that can cause (***upward arrow***) and several that might reduce (***downward arrow***) an acute exacerbation of risk.

Source. Adapted from Gunderson JG, Links P: *Borderline Personality Disorder: A Clinical Guide*, 2nd Edition. Washington, DC, American Psychiatric Publishing, 2008. Used with permission.

diminish their own anxiety. Moreover, many health care systems lack the intensive outpatient or residential levels of care, thereby forcing clinicians to use hospitals to manage self-endangering behaviors.

Invite the Patient to Tell You How You Might Help

See "Basic Therapeutic Approach," subsections "Be Active, Not Reactive" and "Change is Expected," in Chapter 2

Asking a patient to tell you how you can help underscores the patient's need to be active on his or her own behalf (reinforcing the sense of agency and the need to think) and discourages the use of suicidal or self-harm threats as a way to escape, avoid, or externalize responsibility. Do not expect patients to welcome this request; they may refuse or be unable to give meaningful

TABLE 6–3. Borderline personality disorder (BPD) comorbidity: which disorder is primary?

Disorder	% in BPD	% BPD in other disorders	BPD primary?	Reason
Major depressive disorder	50	15	Yes	Will remit if BPD does
Bipolar disorder	15	15		
Manic			No	Unable to use BPD treatment
Not manic			Yes	Recurrence ↓ if BPD remits
Bipolar II			Yes	Will remit if BPD does
Panic disorder	50	7	Yes	Will remit if BPD does
Posttraumatic stress disorder	30	8		
Early onset (complex)			No	Too vigilant to attach/be challenged
Adult onset			?	Able to use BPD treatment?
Substance use disorder	35	10	No	3–6 months of sobriety make BPD treatment feasible
Antisocial personality disorder	25	25	?	Is treatment for secondary gain?
Narcissistic personality disorder	15	25	Yes	↓BPD response to treatment but improves if BPD does
Eating disorder	20	20		
Anorexia			No	Unable to use BPD treatment
Bulimia			?	Is physical health stable?